

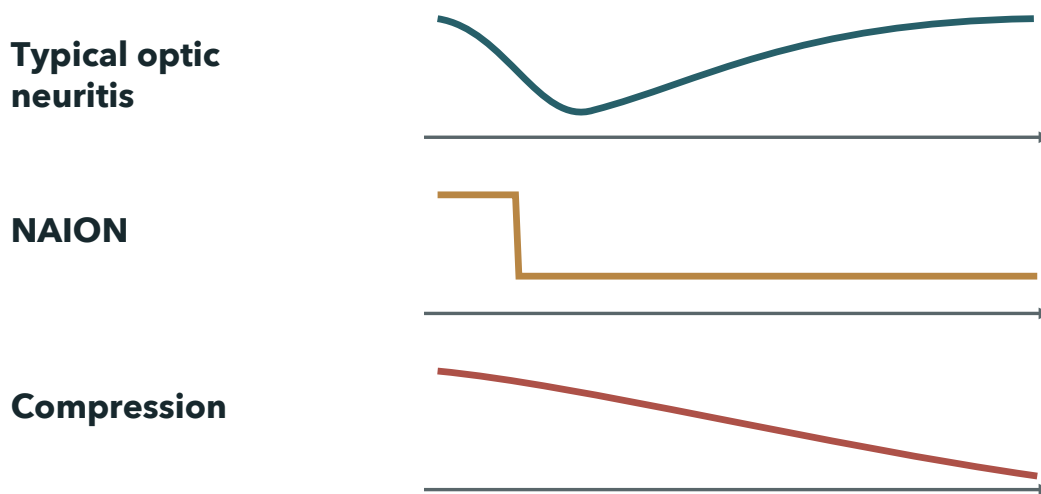
Optic neuropathy: time course and recovery

CLINICAL TYPE B | Focused decisions for M3-M5

The trajectory changes the work-up

Three useful visual trajectories

QUALITATIVE VISUAL FUNCTION OVER TIME



Original qualitative schematic. No numerical prognosis or fixed recovery deadline is implied.

Localise, then use the time course

Finding	Clinical inference
Subacute painful loss	Typical optic neuritis is possible; MRI and specialist assessment also exclude mimics.
Abrupt painless loss	Consider NAION or other vascular disease; urgently assess GCA when indicated.
Steady progressive loss	Colour loss, RAPD and pallor require a compressive/infiltrative work-up; MRI brain/orbits with contrast.

References NANOS: Optic neuritis | NANOS: NAION prognosis and risks | AAO: Compressive visual-field defects

What treatment can realistically change

Counselling after a confirmed diagnosis

Diagnosis	Expectation
Typical demyelinating optic neuritis	Steroids can speed recovery without improving the ultimate average visual outcome. Atypical severe/bilateral disease needs different assessment.
NAION	Permanent field/acuity loss may remain; no routine restorative therapy is proven. Review vascular risks and sleep apnoea.

Keep function in the follow-up

- 1** Record acuity, colour, pupils and reliable fields; a normal-looking disc does not exclude retrobulbar disease.
- 2** Assess both eyes and practical visual needs. Persistent or progressive loss warrants reassessment of the diagnosis.
- 3** Retinal oedema treatments do not automatically treat optic-nerve infarction. Match treatment to the injured tissue.